

American Oncology Institute — \$100–150M Full Acquisition

AOI has built a Tier-2 focused Oncology Services Model with 17 non-metro oncology units (mix of standalone cancer centres and shop-in-shops)

- Deal Size** — ₹1,400–1,600 cr (~\$140–160M) for 100% equity; ~₹500 cr (~\$50M) of debt on the balance sheet; net cheque ~₹900–1,000 cr (~\$90–100M). *Five bidders in Round 2; Deal runners are Alvarez & Marsal (Mohit Khullar, MD, is the key person involved)*
- History** — Cancer Treatment Services International (CTSI) started AOI in 2013. TPG Growth invested in 2016 at ~\$33M for 65%; and sold 100% to Varian (Siemens) in 2019 at \$283M (~8x revenue); AOI now treated as non-core within Siemens Healthineers (post-Varian) and being offloaded at ~3x revenue.
- Revenue** — AOI - FY25 ₹437 cr (~\$44M), FY24 ~₹375 cr est., FY23 ~₹315 cr est. *AmPath* (diagnostics arm): FY25 ₹106 cr (~\$11M). Consolidated network per A&M: FY26E ₹600 cr (~\$60M), FY27P ₹824 cr (~\$82M), FY32P ₹1,639 cr (~\$164M) — ~15% CAGR.
- EBITDA & margins** — From filings, FY23 revenue +18% but operating profit is -16.7%, indicating that the Shop-in-Shop model has thin margins. FY27P EBITDA ₹85 cr (~10.3%); FY32P ₹329 cr (~20%; ~31% CAGR) For comparison, HCG runs at 17.5% margin.
- Business Model** — Hybrid: comprehensive cancer centres + capital-efficient Shop-in-Shop (SIS) overlays inside partner hospitals and medical colleges. Long-tenor (15–20 yr) partner contracts. 60%+ doctors on fixed payout with ~18% doctor cost ratio (vs ~20% industry). Diagnostics chain (22 NABL labs + 600+ collection centres, run via AmPath) currently a parallel revenue stream.
- Units** — 17 centres (16 in India + 1 Sri Lanka). 942 beds; 95 oncologists; 19 LINACs and 12 PET-CTs.

Business Line Overview

- 3 business lines :
 - AOI — 17 specialised oncology units, the flagship (*3rd largest specialised oncology platform by revenue and 2nd largest by centre footprint*);
 - Citizens Specialty Hospital, a multi-specialty hospital in Hyderabad; and
 - AmPath Diagnostics, a 22-lab and 600-collection-centre diagnostics chain.

Deal Pros

- Instant scale and oncology brand in India, which would otherwise take 2-3 years to build** — AOI has good brand recognition in tier-2 markets and provides immediate entry into the Oncology space.
- Cancer network positioning** - Burjeel Cancer Institute + VPS Lakeshore + Qutub Delhi + AOI is a credible India oncology story to redefine the overall brand positioning and from a signalling lens.
- Patient volume and clinical depth** — Large patient and doctor base. 72k chemo cycles and ~8k radiation patients, with a large BMT program at Hyderabad, Vijayawada. They also claim to have 63% cash + insurance patients (vs HCG 53%).
- Three-hub cluster fit** — This tier-2 market presence could provide us an opportunity to build around three geographically defined clusters, each anchored by a VPS hub: Delhi PMC (North — 8 AOI feeders), Hyderabad CoE (AP/Telangana — 2 feeders), and Lakeshore Kochi (South — Coimbatore).
- Capex cycle already paid for** — ₹740 cr (~\$74M) gross block; fixed-asset turnover 1.0x. ₹410.94 cr (~\$41M) of paid-up capital in CTSI to fund the 9 LINACs since FY23 and 4 more in the ramp stage; *Next-five-year capex requirement is only ₹213 cr (~\$21M)*.
- Day-one referral pipeline for Qutub Delhi** — **There are 8 AOI feeders around Delhi** which gives us an immediate referral pathway. AOI Gurugram is the first AOI centre with both NABH + JCI dual accreditation.
- AmPath + OneDx lab footprint** — 22 NABL labs + 600+ collection centres is a good way to quickly build a lab pipeline.

Deal Cons

- This is a down round.** 8x revenue (2019) → ~3x today. We think this is because margins in a SIS model are very tight.

- **Fragile SIS economics.** Onco overlays in partner hospitals usually work until the host gets greedy on revenue share. 15–20-year contracts help, but change of control will have a renegotiation risk. FY27P 10.3% EBITDA margin is very low, meaning that this will need multi-year execution and more money to turn it around.
- **Competitive auction.** Good to anticipate that there will be other PEs involved in the second round of bidding (Five bidders projected).
- **No real-estate ownership.** Key downside is that we will not be full owners of the centre, which will impact how we view long term capex commitments.
- **Integration complexity.** 11 states, 2 countries, 4 ramp-stage centres below breakeven, modality mix biased to low-margin medical (62%) at ~25% LINAC utilisation.
- **BMT footprint loss.** Two of AOI's four BMT-capable centres (Nagpur, Imphal) sit in our divestiture set. We retain BMT only at Hyderabad and Vijayawada. If we divest, we lose Manipur's only BMT facility and something to further examine the strategic loss against the geographic-isolation argument.
- **Low revenue per centre.** ~₹26 cr/centre across 16 hospitals vs ~₹100+ cr/centre for HCG. Even with low Tier-2 ceilings this signals that there is a volume saturation in micro-markets which can cap the same-centre growth runway.
- **Tier-1 counterfactual untested** — ~₹1,000 cr could instead build or buy 2–3 onco hubs in Tier-1, which could deliver similar or higher revenue per year immediately.

Comparable Deals and Price point

- **AOI / Varian (2019):** \$283M, 8x revenue;
- **HCG (May 2026):** 17.5% EBITDA margin with an EV/EBITDA ~23x. AOI's FY27P implied at ₹1,400 cr EV / ₹85 cr EBITDA ~ ~16–17x.
- **Indian healthcare-delivery comps:** Multi-specialty hospital EBITDA multiples 15–20x; specialty pure-plays usually command a premium.
- **Implied multiple on this deal:** At ₹1,400 cr EV → ~1.7x FY27P revenue, ~16x FY27P EBITDA.

If we think this is worth it, how to make it work?

AOI Network Turnaround Strategy

- **Modality mix shift** from 62/25/13 (med/rad/surg) toward 50/35/15 since radiation drives higher revenue per episode and better margins once LINAC utilisation crosses 50% (currently ~25%). No need to spend further on capex immediately.
- **Case mix elevation** toward stage III/IV, rare tumours, relapsed/refractory cases via tumour board visibility and structured engagement with local oncologists, GPs and surgeons.
- **Diagnostics integration with OneDx to get scale benefits** — Closer integration with the 22 labs will ensure we are able to capture investigation revenue and stop external leakage.
- **Group procurement** — consolidate chemo drugs, supportive care, radio, pharma across 13 retained centres + VPS Lakeshore. This is the single largest cost line, so can increase margins if the material price reduction is possible.
- **Tariff benchmarking and rationalisation** — many AOI tariffs underpriced vs local private competitors, particularly radiation fractions, PET-CT, day-care chemo, which can be re-branded and repositioned.
- **TPaaS network-wide** — extend AOI's centralised treatment planning platform to VPS Lakeshore and Qutub. This will mean lower per-fraction planning cost, hub-level QA of complex cases.
- **Network tumour board** — weekly virtual MDT converts ad-hoc referrals into a protocol-driven, measurable pipeline into VPS Lakeshore and Qutub Delhi. Also this is the primary clinical-quality signal in local markets.
- **Shared subspecialist workforce** — VPS Lakeshore and Delhi senior subspecialists rotate to AOI centres for surgical oncology, haemato-onco, interventional radiology; AOI clinicians rotate to hubs for exposure. This reduces full-time staffing need.
- **Clinical trials satellite enrolment** — Lakeshore / Delhi anchor trials; AOI centres enrolled as satellite sites. Per-patient trial revenue, academic engagement (top retention driver for senior oncologists), which can be a local market quality signal.

Clusters

- **North cluster → Delhi PMC (hub):** Gurugram, Hisar, Ludhiana, Jalandhar, Jammu, Sri Ganganagar, Kota, Aligarh (8 feeders).
- **AP/Telangana → Hyderabad CoE (hub, 188 beds):** Guntur, Vijayawada (2 feeders).
- **South → Lakeshore Kochi (hub):** Coimbatore (1 feeder, ~185 km from Kochi, within established travel tolerance).
- **Divest:** All four divested as integrated onco-diagnostics units (centre + lab + collection + imaging) and proceeds offset acquisition.
 - **Nagpur, Raipur** — referral orientation toward Mumbai, Hyderabad, Tata Memorial; no viable upstream pathway to our hubs.
 - **Imphal** — genuine NE oncology need but geographic isolation makes hub-spoke transfer operationally impractical.
 - **Colombo** — cross-jurisdictional regulatory and governance complexity disproportionate to strategic value.

How to make the network play happen

1. Route through VPS Lakeshore — Burjeel infuses ~₹900–1,000 cr of primary equity into Lakeshore, and Lakeshore acquires AOI. *Lakeshore is an established Indian operating company and can absorb AOI cleanly, which also keeps the India oncology story under one identifiable hub. This will also have simpler FDI and CCI path than a new SPV while diluting existing Lakeshore minority holders.*
2. Route through VPS Healthcare India — Burjeel acquires AOI directly via VPS Healthcare India with VPS Lakeshore subsequently rolled into the same entity. *This is a cleaner hold co architecture if Burjeel intends to add further India assets beyond AOI, consolidates Lakeshore, and add any future acquisitions under one balance sheet.*

How much capital — total commitment, working estimate

- **Net acquisition consideration:** ~₹900–1,000 cr (~\$90–100M), after ₹500 cr (~\$50M) debt and divestiture proceeds from Nagpur/Raipur/Imphal/Colombo.
- **Year 1–3 reinvestment (post-close):** ~₹250–400 cr (~\$25–40M). Working-capital normalisation, ramp losses on the 4 sub-breakeven centres, modality-shift capex, group procurement system, JCI accreditation rollout, integration.
- **Lakeshore primary infusion:** ~₹1,000 cr (~\$100M).
- **Total Burjeel commitment:** ~₹2,100–2,400 cr (~\$210–240M).

Key Financials

Metric (₹ cr)	CTSI			Consolidated network		
	FY23	FY24	FY25A	FY26E	FY27P	FY32P
Revenue — CTSI/AOI (₹ cr)	~315	~375	437	—	—	—
Revenue — AmPath (₹ cr)			106	—	—	—
Revenue — consol. (₹ cr)	—	—	—	600	824	1,639
Revenue (\$M @ 1:100)	~32	~38	~54	~60	~82	~164
Revenue YoY (AOI)	+18.1%	~+19%	~+17%	+37%*	+37%	—
Op margin (MCA)	0.25%				~10.3%	~20%
EBITDA (₹ cr)					85	329
Paid-up capital	—	—	₹410.94 cr	—	—	—

Sources — A&M Documents, Tofler, Traxcn

Team and Key Doctors

CTSI / AOI corporate leadership

- Dileep Mangsuli — Chairman, CTSI. 3 decades in healthcare; came up through Siemens Healthineers ecosystem
- Harish Trivedi — CEO, CTSI South Asia (appointed Mar 2023; prior: Regional CEO at Care Hospitals). Runs all three business lines
- Atul Rastogi — CFO; Faisal Siddiqui — COO (Processes); Hanuman Jayaram — CTO; Dr Saino Thomas — Group CNO.
- Medical quality: Dr Vibhor Gupta (AOI Group Quality); Dr Mir Raza Ali Khan (Multi-Speciality Group Quality).

Marquee clinicians — Hyderabad CoE and Delhi PMC anchor faculty

- **Hyderabad CoE**
 - Dr Mallik Singaraju — Senior Consultant Radiation Oncology (~2 decades); ramp lead at CoE.
 - Dr Pratap Varma Penmetsa — Consultant Surgical Oncology (18+ yrs).
 - Dr Harjot Kaur Bajwa — Consultant Radiation Oncology & Brachytherapy (10+ yrs).
- **Delhi PMC cluster (Gurugram hub + feeder centres)**
 - Dr Babita Singh — Sr Consultant, Radiation Oncology, AOI Gurugram (15+ yrs); MBBS Lady Hardinge, MD Radiotherapy MAMC
 - Dr Sandeep Ramawat — Consultant, Medical Oncology, AOI Gurugram; MBBS MAMC, DM Medical Oncology Gujarat CRI.
 - Dr Neeraj Bishnoi — Zonal Director, AOI North (Punjab, J&K, Himachal); operations spine of the 8-feeder Delhi cluster. Background: Paras Hospitals (Director, Facility; GM Operations & Quality),

Network total: 95+ oncologists — 3rd largest among India's specialised chains (HCG 400, Omega 120,)

More Due Diligence Needed

- Centre-level P&L with disaggregated diagnostics revenue per site, and validate retention/divestiture classification, price the divested units, identify underperformers in the retained set.
- SIS partner contracts — change-of-control provisions, assignment rights, exclusivity clauses, revenue-share economics.
- NABL accreditation structure — entity-level vs site-specific — for clean transfer mechanics in divestiture transactions.
- LINAC and PET-CT ownership vs operating-lease confirmation for all centres.
- Regulatory mapping across 11 states — licence transfer requirements, expected timelines, CCI clearance.
- Clinical leadership retention assessment at Hyderabad, Coimbatore, North India cluster.
- FY19–26 historical revenue and EBITDA trajectory to understand why the business has not compounded. Same-centre vs new-centre growth split. Bridge from \$283M to ~\$170M.
- Real estate ownership map across all 16 centres — what we lease, from whom, lease tenor, renewal terms.
- Working capital normalisation — receivables ageing, particularly government-scheme exposure.
- Independent commercial diligence on A&M's Phase I tariff-optimisation lever — magnitude, timing, attainability.

Sources

1. Project Sunrise Investment Overview, Alvarez & Marsal India, May 2026;
2. Bain India HC PE Deals Database, 2023
3. AOI corporate website: americanoncology.com/disclaimer (CTSI corporate structure); /leadership (CXO bench); /our-doctors/Hyderabad (marquee clinicians); /media (BMT and accreditation milestones).
4. Financials: Cancer Treatment Services Hyderabad Pvt Ltd — MCA filings via Tofler & Tracxn (CIN U85110TG2006PTC051690); American Institute of Pathology & Laboratory Sciences Pvt Ltd — Tracxn FY25 profile; HCG Q3 FY26 results via TipRanks;
5. News & accreditation: CancerRounds AOI Gurugram NABH+JCI; DrugTodayOnline & Medicircle on AOI bloodless BMT, Manipur first BMT; IndianPharmaPost on AOI BMT programme expansion; BioSpectrum Asia on CTSI leadership.
6. Tofler CTSH financials